



Building evidence for sexual and reproductive health services and rights

Impact report 2018/2019



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ABBREVIATIONS

AGYW	Adolescent Girls and Young Women
ANC	Antenatal Care
CSOs	Civil Society Organizations
DIC	Drop-In Centre
FSW	Female Sex Worker
FTYM	First Time Young Mother
HIV	Human Immunodeficiency Virus
ICRHK	International Centre for Reproductive Health-Kenya
IDPs	Internally Displaced Persons
KDHS	Kenya Demographic and Health Survey
KNBS	Kenya National Bureau of Statistics
KNCHR	Kenya National Commission on Human Rights
KVP	Key and Vulnerable Populations
LGBTI	Lesbian, Gay, Bisexual, Transgender and Intersex

MSM	Men who have Sex with Men
NASCOP	National Aids and STI Control Program
PLE	Public Legal Education
PLHIV	People Living With HIV
PMA 2020	Performance Monitoring Accountability 2020
PrEP	Pre-exposure prophylaxis
PSK	Population Services Kenya
SNA	The Social Network Analysis
SRH	Sexual and Reproductive Health
SRHR	Sexual and Reproductive Health Rights
TBA	Traditional Birth Attendant
UNFPA	United Nations Population Fund
WHO	World Health Organization

ICRHK AT A GLANCE

International Centre for Reproductive Health (ICRH)-Kenya (ICRHK) is an independent, local non-governmental organization (NGO) established in the year 2000. ICRHK is affiliated to the ICRH global group, with independent country offices in Belgium and Mozambique.

ICRHK currently has a total of 81 staff engaged in 12 different research and intervention projects. The staff are experienced in program design and implementation, and carry out field large-scale community interventions for behaviour change in high-risk groups. The teams are multidisciplinary composed of researchers, clinical teams and the social scientists. ICRHK interventions

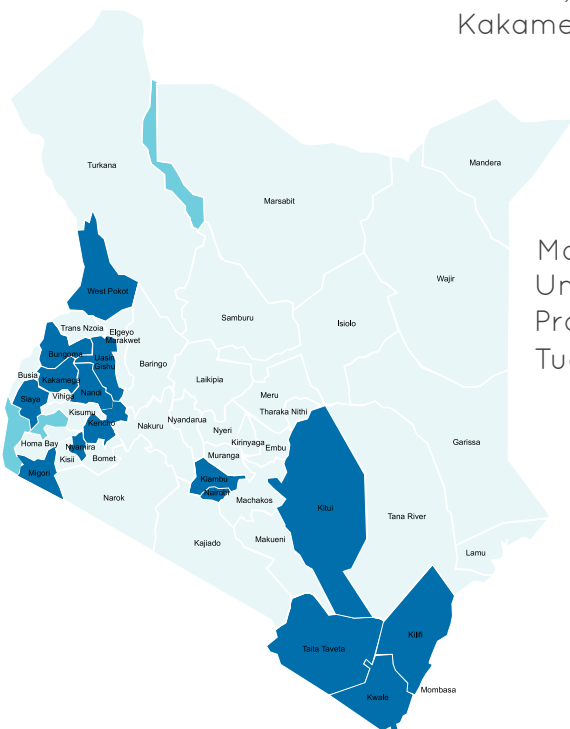
are based on the best available scientific evidence which are critically monitored by scientists. Both the research and the interventions undertaken are shared and embedded in continuous dialogues and feedback with the communities concerned. Over the last 18 years, ICRHK has partnered with various organizations to implement high-quality sexual and reproductive health (SRH) programs. Through this collaboration and experiences, ICRHK has established robust and efficient systems for programmatic and financial management of donor funding.



Geographical Coverage

ICRK Kenya programs are mostly in the coastal area. From 2000 to 2014, ICRH projects were within Mombasa, Kwale, Taita Taveta and Kilifi Counties. From 2014, ICRH expanded to 11 other Counties, Nairobi, Kitui, Nyamira, Siaya, Kakamega, Kericho, Uasin Gishu, Kiambu, Migori, Bungoma, Nandi, West Pokot.

ICRH offices/headquarters are in Mombasa, at the Technical University of Mombasa, Prof. Ali Mazrui Building in Tudor.



ICRH Approach

ICRH designs and implements innovative evidence-based and cost effective interventions based on research in the field of sexual and reproductive. The interventions are implemented with a continuous engagement of stakeholders. The engagement ensures delivering on promises, engaging in policy dialogues that are rooted in scientific evidence and promoting positive change for outputs that are locally and globally relevant in sexual reproductive health.



ICRHK Core Values

ICRHK is guided by the following core values:

Respect for human life | Innovation and creativity |
Integrated approach | Teamwork | Professionalism, |
Partnership | Commitment to continuous learning |
Integrity, Fairness, Transparency and Accountability.

Our vision

“To be a center of excellence in sexual and reproductive health research and interventions”

Our mission

“To contribute to improved health status of Kenyans by designing and implementing innovative, evidence-based and cost-effective interventions and conduct research capable of influencing public policy and practice in the field of sexual and reproductive health”



MESSAGE FROM PROF. MARLEEN TEMMERMAN

I am glad to present to you the ICRH Kenya 2018/2019 impact report. Through this report, we take stock of how our work has impacted the communities that we serve, and look forward to continuing this service. I am particularly excited this year because the global ICRH community turns 25 years old! For one quarter of a century, we have striven to provide high-quality sexual and reproductive health services to marginalized and underserved communities, conduct high-quality cutting-edge research to inform local and global policies and from these studies, built projects that leave significant impact on communities. To provide a brief history, my colleagues and I in Belgium es-

tablished the International Centre for Reproductive Health as a center for excellence in Sexual and Reproductive Health (SRH) in 1994. We were inspired by the deliberations during the conference and the rallying call at the conclusion of the International Conference on Population and Development (ICPD) in Cairo, Egypt in 1994. Interestingly, it is also from this conference that the resulting program of action served as the steering document for the establishment of the United Nations Population Fund (UNFPA), the United Nations' sexual and reproductive health agency. Birthed from the same process, ICRH and UNFPA have collaborated to research and run programs on sexual and gender-based violence, mater-

nity shelters, adolescent and young people's health, and SRH for key populations.

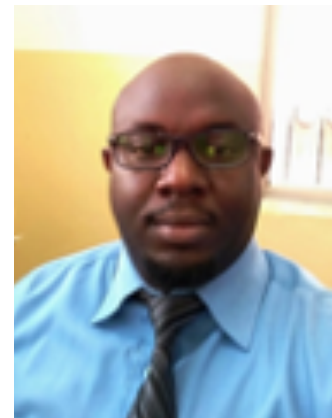
Over these 25 years, we have a well-built ship. We have trained hundreds of local staff in research and project management to run this ship. We look forward to the future with hope and anticipation of even more successful times ahead. We are eager to contribute to improved maternal, neonatal and children's' health, reduced sexual and gender based violence and HIV/AIDS in Kenya. We are eager to lend our experience, expertise and enthusiasm in advocating for progressive sexual and reproductive health policies and laws that will ensure equity, equality and better access to SRH services.

MESSAGE FROM THE COUNTRY DIRECTOR

I am delighted to present the International Centre for Reproductive Health Kenya (ICRHK) Impact Report for 2016 to 2018! This report highlights the various projects, studies and trials ICRHK has undertaken in the past four years leading up to the end of 2018. Here you will find the impactful ways ICRHK's work has had on thousands of beneficiaries and on the wider Kenyan health sector.

Having been operational since 2000 in Kenya, ICRHK is steadily growing, taking up large-scale interventions and thus implementing high-quality sexual and reproductive health (SRH) programs, targeting the most vulnerable and key high risk populations. Our work has touched on the lives of many,

while at the same time blazing ahead to change and improve the SRH environment in the country. As we continue to grow, we continue to learn, and this has only been possible through collaboration, hard work, perseverance and determination of the various stakeholders involved, I would like to take this opportunity to thank our donors whose financial support continues to ensure that our good work flourishes and has the greatest positive impact possible on the health of Kenyans. Together with that, the direction and support of the ICRHK board of directors has held steadfast in keeping the organization committed to its vision and mission. I cannot forget to mention the hard work and dedica-



tion of the ICRHK staff all over Kenya, who with their unwavering loyalty, continue to be at the frontline, fighting to ensure Kenya's SRH services attain the highest standard possible. Finally, I would like to extend my sincerest gratitude to our various beneficiaries, who have warmly welcomed us into their communities with great willingness to take part in ICRHK's projects, while continuing to play an active role in the work being done.

Looking to the future, our mission is stronger than ever, and as you read through this impact

report, I invite you to reflect on the wide range of interventions, research and collaboration ICRHK has been and continues to be involved in throughout the years. Whether discovering us for the first time or you are a long-term friend, we hope you stay with us for many years to come.

Enjoy the reading!



Dr Griffins Manguro
ICRHK Country Director

1

ONGOING INTERVENTION PROJECTS



Linkages across the Continuum of HIV

Services for Key Population Affected by HIV

(LINKAGES) Project

Kenya's widespread HIV pandemic is varied amongst the general populace, with key at-risk groups such as men who have sex with men (MSM), women, sex workers (male and female), and people who inject drugs being more vulnerable to infection.

Estimated prevalence of HIV:



4.8% among the general population



29.3% among female sex workers (FSW)



18.2% among men who have sex with men (MSM)

To combat this perturbing epidemic, ICRHK through the USAID-funded Linkages across the Continuum of HIV Services for Key Populations Affected by HIV (LINKAGES) Project, has implemented a comprehensive HIV prevention, care and treatment program, since 2016, targeting sex workers and MSM, in Mombasa, Kwale and Taita Taveta Counties in Kenya.



Peer supervisor demonstrating use of female condom to peers at a Mangwe

Fighting HIV through strategic, targeted and accessible care and treatment

As the LINKAGES implementing partner, ICRHK achieved the following by 2018:

1

201 Integrated Clinical Outreaches:

- **8,186 FSWS** and **1,190 MSM** received HIV Testing Services (HTS)
- **61 FSWS** and **17 MSM** diagnosed HIV positive

2

Drop-in & Service Centres:

- **3,621 FSWS** and **1,446 MSM** received HTS at drop-in centres
- **52 FSWS** and **5 MSM** diagnosed HIV

3

Hotspot Based Peer Education

- Ongoing use of mobile technology (Ujumbe SMS & WhatsApp) to reach more peers with information by the peer educators and outreach workers

4

Sexual reproductive health (SRH) services e.g. Sexually Transmitted Infections (STI) Screening and Treatment, family planning & cervical cancer screening:

- **4,106 FSWS** were screened for STIs with **199 FSWS** diagnosed and treated,
- **604 MSM** were screened for STIs with **32** diagnosed and treated

5

Structural interventions including Sexual and Gender-Based Violence (SGBV) management & follow-up:

- **101 SGBV** cases (**88 FSWS** & **15 MSM**) reported and addressed



Did You Know?

Current studies show that **PrEP has up to 90% effectiveness** in preventing HIV among people at continuous high risk of HIV infection if taken daily!



Key Achievement of Jilinde at the Coast

Starting in July 2016, a total of **4,317 FSW** and **917 MSM** have taken up oral PrEP in ICRHK supported sites by the end of 2018!

Bridge to Scale (Jilinde) Project

The Bridge to Scale Project (“Jilinde” translated from Swahili—to protect yourself) is a pioneering HIV prevention scale-up that aims to demonstrate and document an acceptable, affordable, and replicable approach to launch and maximise the use of oral pre-exposure prophylaxis (PrEP) and reduce HIV incidence among key and vulnerable populations (KVP) [such as adolescent girls and young women (AGYW), female sex workers (FSW), men who have sex with men (MSM) including male sex workers, & people who inject drugs] when implemented at a population level in “real life” routine service delivery.

Funded by the Bill and Melinda Gates Foundation through the Jilinde Consortium (JHPIEGO, Population Services Kenya (PSK), National Aids and STI Control Program (NASCO), ICRHK and Avenir Health), this timely 4-year project targets over 20,000 people who are at high-risk of HIV infection in Kenya.

Impact of Jilinde on Key and Vulnerable Populations

From its inception, Jilinde has proven that peer education and continuous demand creation efforts significantly play a role in ensuring PrEP up-take amongst peers. Remarkably, more FSWs compared to MSM continue to use PrEP for longer durations (more than one month), with 38% PrEP continuation rates recorded in the first month among FSWs compared to no continuation rates among MSM. Strategic implementation of the oral PrEP scale-up has indicated a fundamental need to innovate and strengthen existing PrEP continuation services to guarantee an increase in KVPs’ use of PrEP for longer periods of time, that is, beyond the third month.



"Jilinde" the ICRHK Way

ICRHK in liaison with NASCOP, is scaling up PrEP in Kenya through Jilinde to ensure demand creation and service delivery. Focusing on the coastal region, ICRHK continues to create PrEP awareness & demand by:

- Peer educators conducting hotspot group and one on one sessions
- PrEP initiation and refill services at the Drop-in Centres (DIC) & at Clinic Outreaches
- Monthly PrEP support groups i.e. PrEP clubs
- Dissemination of glow-in-the-dark bags
- Through the use of goodie bags among FSW to strengthen PrEP continuation



James Mwaughanga in Mombasa, cannot contain his excitement as he shares monthly PrEP data with the Jilinde team. "Thank you so much for Jilinde Data System, it has made my work a lot easier", says James. He is a clinician at Reach Out Trust Center drop in center in Voi, Taita Taveta County. James discloses that he had very little time recording client data due to the high work load and targets.



*Every day in developing countries, an estimated **20,000** girls younger than 18 give birth" (UNFPA, 2017)*

First Time Young Mothers

(FTYMs) Project

According to a Kenya Demographic and Health Survey (KDHS) 2014 survey, nearly one in five girls aged 15-19 years in Kenya, has begun child bearing or already has a child (Kenya National Bureau of Statistics, 2014). With adolescents aged 10 to 15 years comprising 24% of the Kenyan population and a prevalent high adolescence pregnancy rate in the country, it is not surprising that approximately 13,000 teenage girls drop out of school every year due to pregnancy.

Kilifi County in the coastal region of Kenya, has one of the highest pregnancy rates at 22% among 15-19 year old girls. As such, ICRHK through UNFPA funding, has implemented the First Time Young Mothers (FTYMs) project in Mtwapa Town, Kilifi. This indispensable project aims at reducing the high teenage pregnancy prevalence in Kilifi by:

- Providing accessible sexual and reproductive health (SRH) services
- Optimizing SRH delivery services
- Documenting and sharing lessons learnt among the FTYMs and other stakeholders

The underlying drivers of teenage pregnancy include gender inequality, child marriage, poverty, sexual violence, and poor education and job opportunities.



A nurse at the Mtwapa Health Centre sensitizing on breast feeding during a support group meeting for FTYMs

FTYMs' Impact at a Glance by the end of 2018

- 1,400 FTYMs are currently enrolled and actively participating in the project where they are offered SRH services and incentivized to return for review and health management with diapers, babywear, detergents, & sanitary pads.
- 100% child welfare coverage up to 1 year has been achieved among the FTYMs with majority of them enrolled at antenatal care (ANC) clinics.
- 99% hospital delivery achieved following sensitization of traditional birth attendants (TBAs) who refer the girls to the hospitals instead of delivering them at home.
- 90% family planning coverage with majority opting for the 3-month injectable contraceptive Depo-Provera®, which leaves no mark since most of the male partners in the area do not support the use of contraceptives.
- One-year post-delivery follow-up with FTYMs to ensure that they are on family planning and delay a second pregnancy while empowering them with health information.
- Project stakeholders are engaged on quarterly basis to review project progress and address any teenage pregnancy cases that occur in the area.

Female Sex Workers (FSWs) Project in Kilifi and Mtwapa Towns

With over 200,000 sex workers in Kenya (133,675 female sex workers [FSW], 13,019 men who have sex with men [MSM]) as mapped out by National Aids and STIs Control Programme (NASCOP) in 2016, sex workers face immense challenges daily especially when it comes to their sexual and reproductive health (SRH).



- High risks of HIV, STIs infections, unintended pregnancies, gender violence and adverse birth outcomes
- HIV prevalence among FSWs nationally is estimated to be at 29.7%
- Unintended pregnancies for FSWs are similarly high at 25%

In Kilifi County, where the FSWs project is being implemented by ICRHK, there exists a varied HIV prevalence amongst the general population. Currently it is 3.8% with 33,319 people living with HIV (PLHIV) (Kenya HIV Estimates Report 2018) and with an estimated population of 6,696 FSWs (NASCOP mapping 2018), notably with Kilifi and Mtwapa towns having the highest numbers of FSWs in the county. Through the United Nations Population Fund (UNFPA)-funded FSWs project, ICRHK is providing integrated and stigma-free SRH services to FSWs in Kilifi and Mtwapa towns to help mitigate and reduce health challenges faced by FSWs,

Lessons learnt and experiences gained during project implementation are shared among stakeholders aimed at improving other similar projects in the country.

Over 200,000 sex workers in Kenya



133,675
female sex workers
[FSW]



13,019
men who have sex with men
[MSM]

Impact of the FSWs Project by 2018

- Bi-annual hotspot identification and validation to target FSWs beneficiaries:
 - 2 themed hotspot events conducted per year
 - 2,104 FSWs enrolled into the project
- Peer-led community education mobilization:
 - 10 FSWs in Kilifi and 20 in Mtwapa identified and trained to provide health talks
 - Identified and trained 30 peer educators
 - 871,055 male condoms distributed, 645,852 directly to FSWs through peer educators and 225,203 through other outlets like dispensers
- HIV Testing, training & provision of oral PrEP:
 - 1,024 received quarterly HIV testing
 - 51 FSWs diagnosed with HIV and linked for care
 - 450 HIV negative FSWs enrolled oral PrEP
- Sexual reproductive health (SRH) services e.g. Sexually Transmitted Infections (STI)

Screening and Treatment, family planning & cervical cancer screening

- Increased provision of clinical outreach services:
 - 25 outreaches conducted every month
 - 2 in reach activities conducted every month
 - 2 support groups for FSWs living with HIV formed who meet monthly
- Regular trainings for service providers on provision of quality key population (KP) friendly services.
- Continued and regular stakeholder engagements.



Peer supervisor demonstrating use of female condom to peers at a Mangwe

The '160

Girls' Project

Envision a world where a young girl is denied her human right to seek justice after one of the most appalling forms of violence that is rape. This was the case for a young girl in Meru County, Kenya, in 2013, who during a community meeting, stood up and bravely reported that she had been raped and her perpetrator freed. Following this, human rights activists visited the Brenda Boone Hope (Tumaini) Centre in Meru, a safe haven and rescue centre that at the time had 157 girls and



3 boys who had been defiled. Thus the “160 Girls” Project was born! These victims of defilement and rape made legal history with the Kenya High Court ruling that the police treatment of the “160 Girls” petitioners’ claims violated their human rights, and that the police treatment of defilement had created a climate of impunity for defilement, which rendered them indirectly responsible for the harms inflicted by the perpetrators. This set a notable benchmark in protecting girls from rape as the girls/ petitioners secured access to justice for themselves and legal protection from rape for 10,000,000 girls in Kenya.

ICRHK Taking up the Baton with the “160 Girls” Project

“160 Girls” is a legal advocacy initiative that aims to achieve justice for the girls, and protection against rape in Kenya by addressing the root source of the problem relating to police failure to enforce existing laws that prohibit the sexual assault of girls. ICRHK continues to monitor police treatment of de-

The Scourge of Sexual Violence in Kenya



filement cases reported at the Gender based Violence Recovery Centre (GBVRC) based at the Coast General Hospital Mombasa through a standard intake and monitoring forms.

The Approach

- Create Awareness to girl's student regarding defilement, police obligations & community protection
- Mobilize students to report defilement/sexual violence and provide tools to use



The 160 Girls' Justice Club and Paralegal Mary performing Rose Ohon's song on saying NO to rape



- Strengthen outreach and education as undertaken in stage 1 of the 160 Girls Public Legal Education (PLE) initiative
- Establish Justice clubs in primary schools in regions that are hotspots:
- Strengthen school going children's & girls knowledge and capacity on how to identify and report defilement cases and ability to say no to rape
- Enhance collaborative involvement of police officers in the running of justice clubs at schools

The Numbers by end of 2018!

100

Cases Recruited by ICRHK project paralegals

Case Judgement Made

14

13

Cases Still in Court

Cases Still at the Police Station

13

13

Cases who gave wrong phone number and cases with no contact number

Cases where phones were not in service and not being picked up

46

1

Perpetrator released at the Police station

Schools with Established Justice Clubs

6

Commercial Sexual Exploitation of Children (CSEC) Project

Started in September 2016 with support from Kindernothilfe, ICRHK has been implementing Commercial Sexual Exploitation of Children (CSEC) project. The three-year project covered the larger Changamwe and Kisauni Sub-Counties in Mombasa County, Kenya. Its overall objective was to contribute to the reduction of commercial sexual exploitation and sexual abuse of children as well as improve service delivery to the victims through institutional linkages of both children and caregivers.



The Approach

The following interventions were given;



One on one counselling, life skills training sessions, and vocational training.

Identifying and informing caregivers of the children who are in CSEC, community volunteers and different stakeholders including the police, officers from the children department, and the Ministry of Education officers.



Business skills training of caregivers to combat poverty and reduce the chances of their children getting back to CSEC.



Capacity building sessions on child protection with different stakeholders including; local administrators, religious leaders, paralegals, teachers, school peer educators, community volunteers, out of school peer educators and bar owners. These stakeholders are in turn sensitised children and adults in the surrounding communities leading to an increased number of CSEC cases being reported.

Breaking Barriers through Counselling!

Amina, from Changamwe "I come from a poor family and due to our difficult situation and the absence of financial support; I had to get into begging on the streets of Mombasa town and later involved myself in survival sex."

Living with her mother after her parents' separation, Amina (not her real name) succumbed to the life of survival sex to financially support herself and her 3 siblings through school. Lack of financial resources led to Amina not proceeding to secondary school, and she continued to engage in survival sex as her primary source of income.

"I thank ICRHK for giving me psychosocial support, which has made me change my behaviour. After I was identified by a community volunteer, I was enrolled into the program after verification by ICRHK staff. I then went through 6 counselling

sessions. During counselling, I came to realize that survival sex has negative effects like one can contract sexual transmitted diseases, it can affect one's reproductive system and one can even get infected with HIV/AIDS. I became self-aware of what I was doing and I decided to change. This did not happen at once but it took place gradually. I have now stopped survival sex. After stopping, I started selling clothes so as to cater for my needs and those of my siblings.

I then decided to enrol in a secondary adult class so that I could at least complete my secondary education. I would like to complete my secondary education and go to university to do accounting. I also assist in linking other children who are involved in survival sex to ICRHK so that they can get assistance. "



"I thank ICRHK for their support, I am now aware of myself and confident that I can make it in life" Amina, from Changamwe

2018 in summary



Amplify Change

Project

At the national level, the Constitution of Kenya 2010 guarantees the rights of an individual to the highest attainable standard of health, including reproductive health. It underscores the importance of prioritising the needs of vulnerable and marginalised groups in provision of health care. In addition to the Constitution, Kenya has a number of Acts of Parliament that seek to promote and protect sexual and reproductive health rights (SRHR), which include the Sexual Offences Act 2003, the Children's Act 2001, Prohibition of Female Genital Mutilation Act 2011 among others (Kenya National Commission of Human Rights [KNCHR], 2012).

Inspiring Change through the Community

With the prevalence of teen pregnancy and the HIV epidemic not only in the coastal region, but the

whole of Kenya, ICRHK through the Amplify Change project aims to empower people to realize their sexual and reproductive health rights. Establishing the project in Kilifi, Kwale and Taita Taveta Counties, was done through improved quality of data and analysis that allows advocates to use evidence to support their work, while encouraging increased participation of leadership and representatives from marginalized groups such as adolescents and youth; persons with disabilities; people living with HIV/AIDS; refugees and Internally Displaced Persons (IDPs); Lesbian, Gay, Bisexual, Transgender and Intersex (LGBTI) community; and sex workers.

ICRHK continues to implement and empower individuals about their SRHR by:

- Training and sensitization on sexual and gender-based violence (SGBV) with local motorcycle ("boda-boda") and taxi operators; journalists; senior police officers religious leaders from different denominations;





- Setting up of the Mombasa County SGBV Technical Working Group.
- Strengthened capacity for civil society organizations (CSOs) to advocate for SRHR.
- Publishing of the 'Facing Violence' booklet a multimedia project to unveil SGBV issues in Kenya (Read more about Facing Violence at: <http://icrh.org/sites/default/files/FacingViolence.pdf>)
- Locally develop new ideas implemented and lessons learnt shared.

Do You Know Your Sexual And Reproductive Health Rights?

- Addressing gender-based violence, including sexual violence and female genital mutilation.
- Addressing the causes of unsafe abortion including decriminalization of abortion.
- Challenging stigma, discrimination,
- The right to fight against attitudes and laws that undermine human rights, including on grounds of gender or sexual orientation of LGBTI individuals.
- The right to improved sexual health of young people and girls, including comprehensive sexuality education and addressing child and early marriage.
- The right to access to comprehensive reproductive health services for socially and economically marginalized and vulnerable groups.
- The right to decide if and when to have children.
- The right to have information about sexual health.
- The right to choose your own sexual partner, and the right to sexual identity free of fear and discrimination.

Sexual Violence

Measurement Study

Sexual violence is a public health concern, a violation of human rights, and a marker of gender inequality. In Kenya, it is estimated that nearly two thirds of girls experience sexual assault. Sexual and gender-based violence (SGBV) is higher among girls and women in rural areas and urban slums. Despite the high prevalence of SGBV, majority of survivors do not report to the police according to recent data obtained from ICRHK SGBV Recovery Centre in Mombasa.

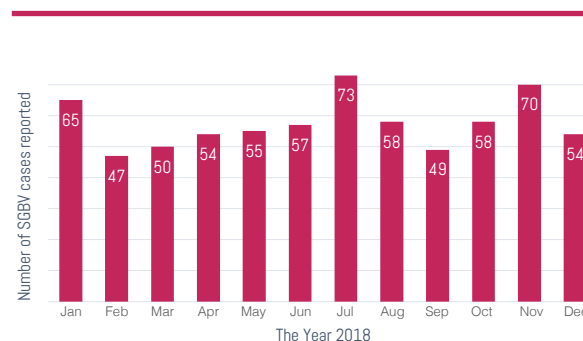
ICRHK is implementing the Sexual Violence Measurement Study to provide an estimate of the prevalence of sexual violence among school-going children, and to assess their attitude and confidence in the police, which determines their reporting these cases. By conducting the study in 4 counties –Kilifi, Kwale, Kisumu, and Homa-bay –the aim is to look for changes in specific attitudes or indicators of

progress in their attitudes towards the police over one year, following a police training exercise to be conducted by the Kenya Police and the Equality Effect Organization.

Currently, the study has already been conducted in one county, Kilifi, while baseline surveys of the SV measurement study in Kwale, Homa-bay and Kisumu will commence early 2020 once schools re-open.



SGBV Cases Reported at the Gender-Based Violence Recovery Centre at Coast General Hospital





What the Numbers Say!

- The study participants were school-going boys and girls aged 12 to 17 years in Kilifi.
- 10 schools and 2 police stations in Kilifi County.
- 94 boys and 56 girls in total were involved in the baseline survey for the study.
- Out of the 150 students, 27% of them personally knew of rape cases while 1% of the students reported they had experienced rape
- 41% of the students believe that if a girl or boy is raped, she/he brings shame to their family.
- When it comes to reporting cases of sexual violence/rape, 44% felt more comfortable confiding in a parent, while only 29% would report the incident to the police.



The ICRHK research team conducting interviews

2

MIXED-METHOD IMPLEMENTATION RESEARCH

(QUALITATIVE AND
QUANTITATIVE STUDIES)





The Boresha Study

ICRHK, launched a three-year (2016-2018) Boresha project that was studying the feasibility of conducting HIV prevention interventions at entertainment venues in Mombasa and Mtwapa towns of Mombasa and Kilifi counties. The project received technical support from ICRH-Belgium and HIV Center for Clinical and Behavior Studies of Columbia University.

Utilising both qualitative and quantitative approaches, Boresha was structured as a multi-level risk-reduction intervention project tailored to the local context which is informed by theory and prior work. It aimed to understand the socio-cultural context of risk behaviour, beliefs/understandings of HIV and risk; barriers to and facilitators of risk-reduction; and responses to intervention messages.

The Study Approach

Formative Study: in depth interviews (IDIs) were conducted with sex workers and their clients of which resulted to two publications and the data analysed has been used in presentations internationally at the AIDS and Impact conferences. Biomarkers study aimed to determine the usefulness of the SPERM HY-LITER and RSID-semen tests as biomarkers for non-condom use for anal sex among sex workers.

The Intervention: Aimed to test the intervention package developed for feasibility, acceptability, as well as participant level of exposure and intervention contamination at control sites via process measures to be used in a future study.

Two independent cross-sectional surveys were conducted with each of the three target populations (FSWs, MSWs, sex workers' clients). The first cross-sectional survey was implemented 2 months before initiating the venue-based intervention to measure the prevalence of potential variables to be used as outcome measures in an actual trial.

Post-Intervention: The second cross-sectional survey was conducted 2-months after the intervention had ended, to assess exposure to the piloted venue-based intervention and acceptability, satisfaction with the intervention, to assess exposure to HIV prevention interventions and assess any major changes in potential outcome measures or contextual factors among a random sample of venue patrons.

The Social Network Analysis (SNA)

Launched on March 2017, Social Network Analysis (SNA) was a qualitative study on the impacts of social norms and social network effects on adoption of modern contraception approaches within the rural parts of Kilifi, Kenya. The project was funded by the Bill and Melinda Gates through George Washington University and implemented by ICRH-Kenya in collaboration with North Western University.

The SNA study was founded on the Performance Monitoring and Accountability (PMA2020) research findings on the level of adoption of modern contraceptives in various counties in Kenya. However, Kilifi County was the preferred project area as it features trends of sites with high, medium, and low levels of prevalence on adoption of modern contraceptives as per the data by PMA2020 ((PMA2020), 2015).



Upon completion in mid-2018, the SNA findings depicted a possibility of failure by many counties to meet the global Family Planning 2020 (FP2020) targets unless a demand creation intervention was made while maintaining a flawless supply chain of the contraceptives for easy access by the communities within the urban and suburb vicinities of the respective counties.



Cultural & Social views and how they shape beliefs on the use of modern contraceptives

The main belief mentioned during the SNA study is that contraception, especially when used at a young age and/ or prior to bearing children, can weaken a woman's uterus or otherwise damage her reproductive system, making it difficult or impossible for her to conceive or to carry a pregnancy to term in the future. The two methods that are most widely used in the study communities—oral contraceptive pills and injectables—were the only methods implicated in the belief.

A total number of 21 focus group discussions (FGDs) and 10 Key Informant Interviews (KIIs) were expected to be conducted in selected areas within Kilifi. The following themes were concluded during the study:

1. The belief that using modern contraception at a young age or before childbirth can make women infertile is widespread
2. According to the above belief, the most commonly used methods in the community were linked to infertility;
3. When women observe other women who cannot get pregnant after using modern contraceptives, they attribute the infertility to the use of contraception;
4. Within the communities, the primary goal of marriage is childbirth and thus community approval is rigidly tied to childbearing; and, therefore
5. The social consequences of infertility are devastating.

Read more on the SNA study:

Sedlander, E., Bingenheimer, J. B., Thiongo, M., Gichangi, P., Rimal, R. N., Edberg, M., & Munar, W. (2018). "They Destroy the Reproductive System": Exploring the Belief that Modern Contraceptive Use Causes Infertility. *Studies in family planning*, 49(4), 345-365.



"She might use family planning and when they are removed from her body, she is not able to conceive. Because those drugs affect the inside and the uterus becomes thin such that it can't carry a pregnancy."

SNA Study Participant

3

RANDOMIZED CONTROLLED TRIALS



Adolescent/Youth Reproductive Mobile Access and Delivery Initiative for Love and Life Outcomes (ARMADILLO)

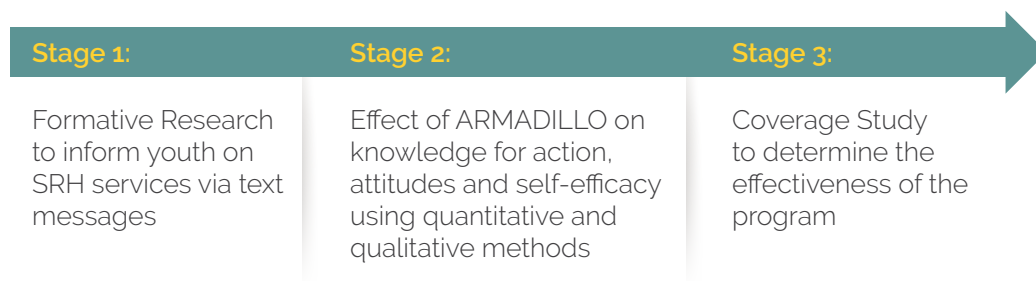
The Kenya government is prioritizing programming among young women through the Kenya Adolescent Reproductive Health Policy (2003). This policy aims at doubling the use of modern contraceptives from 4% to 8% among sexually active adolescents (aged 15-19 years) and from 19.9% to 40% among youth (20-24 years) by 2015. National guidelines for family planning (FP) supports the provision of FP services for adolescents and youth.

Underlying Issues: In order to accomplish these policy targets, strategic actions have been identified for promoting the health of adolescents and youth including the acknowledgement of their right to reproductive services and increasing their access to these services as unintended pregnancy among adolescents is a common public health problem globally, and is associated with significant health risks and social costs

Funded by the World Health Organization (WHO), ICRH-Kenya is implementing Adolescent/Youth Reproductive Mobile Access and Delivery Initiative for Love and Life Outcomes (ARMADILLO) as a multisite study refining the existing mHealth platform in Kilifi and Mombasa Counties by:

- Researching youth learning and information retention following delivery of sexual and reproductive health (SRH) information via mobile phones.
- Determining whether youth given access to SRH information through their mobile phones are more knowledgeable about contraception and better able to dispel contraception myths and misconceptions than those without access to SRH

The ARMADILLO Approach



Notable Impact of ARMADILLO

Stage 1 of the ARMADILLO study established that mobile phones are currently part of young people's lives and reinforced the fact SRH messages delivered via text messages from a trusted source will most likely be acceptable to young people owing to its confidential nature.

In-depth interviews were conducted with a sub-sample of intervention participants at the end of the study, in order to assess the system's usability.



4

MEASUREMENT STUDIES



Performance

Monitoring and

Accountability 2020

(PMA2020)

Maternal and child morbidity and mortality continues to be a public health problem with many sub-Saharan Africa countries. As such, family planning (FP) use remains a central intervention in addressing maternal/child morbidity and mortality as well as HIV/AIDS. In July 2012, the London Summit reinvigorated FP as a health development issue of global importance, particularly in the low resource settings. A significant commitment of resource was made to advance family planning and thereby provide access to modern contraceptives to an additional 120 million women of reproductive age in 69 low in-come countries by 2020.

In the face of such an ambitious objective, there is the need for an effective monitoring system to track annual progress on key indicators around contraceptive demand, supply and use. Performance Monitoring and Accountability 2020 (PMA2020) is a five-year project that uses innovative mobile technology to support low-cost, rapid-turnaround, nationally representative surveys to monitor key indicators for family planning and water and sanitation. ICRHK, the Ministry of Health, National Council for Population and Development, in collaboration with Kenya National Bureau of Statistics, University of Nairobi, Moi University, Technical University of Mombasa and Jaramogi Oginga Odinga University of Science and Technology, lead PMA2020 in Kenya. The project was made possible by generous support from the Bill & Melinda Gates Foundation.



The Key Results of PMA2020/Kenya

Trainings:

- 169 Data Collectors Trained
- 4 staff participated in a data translation workshop in Baltimore
- 3 Staff participated in data visualization and communication training
- 5 staff participated Training Design & Facilitation Bootcamp

Data Collection:

- 1 FP Round of data collection
- 1 Nutrition Round of data collection
- 3 PMA Agile Rounds of data collection

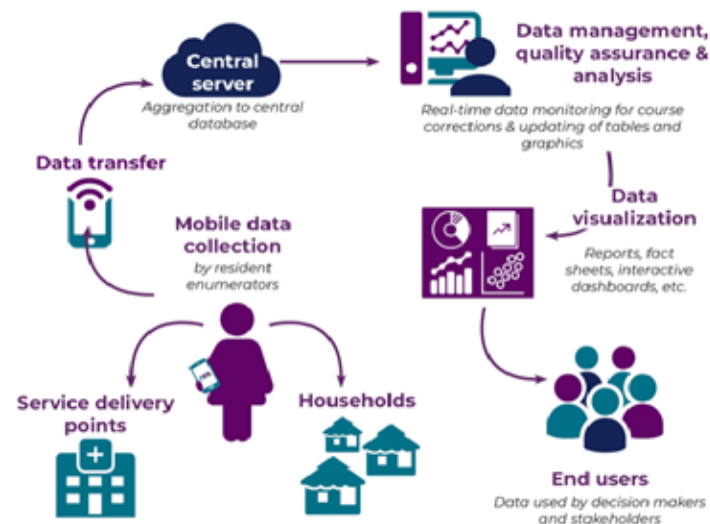
Stakeholders engagement:

- 2 National dissemination Conducted
- 9 County Disseminations conducted
- 2 Costed Implementation Plan (CIP) review meetings (Migori County & Bungoma County)

Data Access and Use:

- 2 Data use workshop conducted
- 1 Data Interrogation workshop Conducted
- 1 Writing workshop Conducted
- 1447 Datasets downloaded
- 2 Masters students using PMA Data
- Engaged 6 Health Journalist in a field work activity to generate data stories

PMA2020's Approach



Access the PMA2020/Kenya Publications at:
<https://www.pma2020.org/research/country-reports/kenya>

Performance Monitoring

and Accountability

(PMA2020/Kenya) Nutrition

The Performance Monitoring and Accountability PMA2020/Kenya Nutrition survey in 2018 used a multi-stage stratified cluster design with urban-rural and 11 selected counties as strata. A sample of 151 enumeration areas (EAs) was drawn by the Kenya National Bureau of Statistics for the Kenya Round 6 PMA2020 family planning survey and these EAs were used for nutrition survey. In each EA, 56 households were randomly selected. The household survey was administered to all consenting households selected.

Twenty-five percent of households were then randomly sub-selected. The female-child questionnaire was administered to all women age

10-49 in sub-selected households, and to caregivers of children under five in all selected households, with one form per child. Data collection was conducted between May and August 2018. The final sample included 8,046 households (95.2% response rate), 4,508 eligible households (56.0% eligible rate), 2,471 females (98.96% response rate), and 4,563 children under 5 (99.8% response rate).

PMA2020/Kenya Nutrition is led by the Ministry of Health in collaboration with International Centre for Reproductive Health Kenya (ICRHK), National Council for Population and Development, and Kenya National Bureau of Statistics. Overall direction and support is provided by the Bill & Melinda Gates Institute for Population and Reproductive Health at the Johns Hopkins Bloomberg School of Public Health.

Access the PMA2020/Kenya Nutrition Report at: <https://www.pma2020.org/research/country-reports/kenya>



PMA Agile

PMA Agile is a component of the Performance Monitoring for Action project and aimed at the subnational level (state, county or city). It builds on the PMA monitoring and evaluation platform and conducts continuous tracking of family planning service delivery and consumption through quarterly public and private health facility surveys and semi-annual client exit interviews. A phone follow-up survey is conducted with consenting female clients four months after their interviews. PMA Agile monitors the urban areas of three counties in Kenya, Kericho, Migori and Uasin Gishu, and is conducted by the International Centre for Reproductive Health-Kenya (ICRHK), in collaboration with The Bill and Melinda Gates Institute for Population and Reproductive Health at the Johns Hopkins Bloomberg School of Public Health.



Breastfeeding Counseling and Support at Four Critical Contact Points

DURING ANC

- 60% of pregnant women received information from ANC provider about how to feed their newborn.

WITHIN 2 DAYS OF DELIVERY

- 81% of women gave birth in a facility.
 - Of those, 59% said a health care worker helped them put the baby to breast the first time.
- 46% of all women received information about feeding their newborn.
- 50% of all women were observed breastfeeding by a health worker to ensure correct technique.

AT VISIT WITHIN 2-30 DAYS OF DELIVERY

- 43% of women received information about feeding their newborn.
- 42% of women were observed breastfeeding by a health worker to ensure correct technique.

AT SICK CHILD VISIT IN PREVIOUS 2 WEEKS FOR BREASTFEED CHILD 0-23 MONTHS

- 61% of caregivers were counseled about continued breastfeeding during illness.

COMPLEMENTARY FEEDING

Complementary Feeding: Appropriately Timed and Targeted Counseling

FOR 0-5 MONTH OLDS

- 33% of caregivers ever received advice on complementary feeding.
 - Of those, 67% received advice in the last month.

FOR 6-11 MONTH OLDS

- 46% of caregivers ever received advice on complementary feeding.
 - Of those, 46% received advice in the last month.

FOR 12-23 MONTH OLDS

- 48% of caregivers ever received advice on complementary feeding.
 - Of those, 26% received advice in the last month.

FOR CHILDREN 0-23 MONTHS WITH SICK CHILD VISIT IN PREVIOUS 2 WEEKS

- 48% of caregivers received advice on continued feeding during illness.

PMA Agile/Kericho	PMA Agile/Migori	PMA Agile Uasin Gishu
1,973 clients (73% female) were interviewed at the 106 public and 98 private facilities.	2,011 clients (75% female) were interviewed at the 101 public and 104 private facilities	1858 clients (80% female) were interviewed at the 100 public and 109 private facilities.
Female clients interviewed at public and private facilities were more likely to choose their contraceptive method themselves (69% and 53%, respectively), than as a decision made with their partner and/or provider	Over 80% of female clients interviewed at public and private facilities report ever being told when to return for a follow-up visit.	Youth clients reported high levels of explanation on how to use contraceptive methods, from private facilities, ranging from health centers (79%), hospitals (78%), to pharmacies (56%).
A relatively high percentage of young clients (18-24 years) report providers explaining how to use contraceptive methods at private facilities. Among public facilities, youth clients reported receiving an explanation about methods primarily from health centres (30%).	More non-contracepting male clients interviewed at private facilities (64%) intend to use family planning in the future, compared to just 20% of those interviewed at private facilities.	Female clients interviewed at both public and private facilities report relatively high levels of satisfaction with such services as clarity of FP information, polite treatment, and range of services compared to other facilities.
Less than 10% of female clients surveyed at private facilities aged 18-24 years and 35 years or older were told what to do if they experienced side effects related to their contraceptive method. These rates were higher among clients of all age groups interviewed at public facilities and those interviewed at private facilities between 25 and 34 years old (53%).	Among female clients interviewed at Migori facilities, 97% consented and completed a follow-up interview four months later.	At the follow-up interview, 50% of female clients were still using the method they reported at baseline, 15% had switched methods, and 13% had stopped using a method; 8% began using a method and 14% remained non-users
Injectables and implants were the most common methods used among clients interviewed at both public and private facilities in all 3 counties!		







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